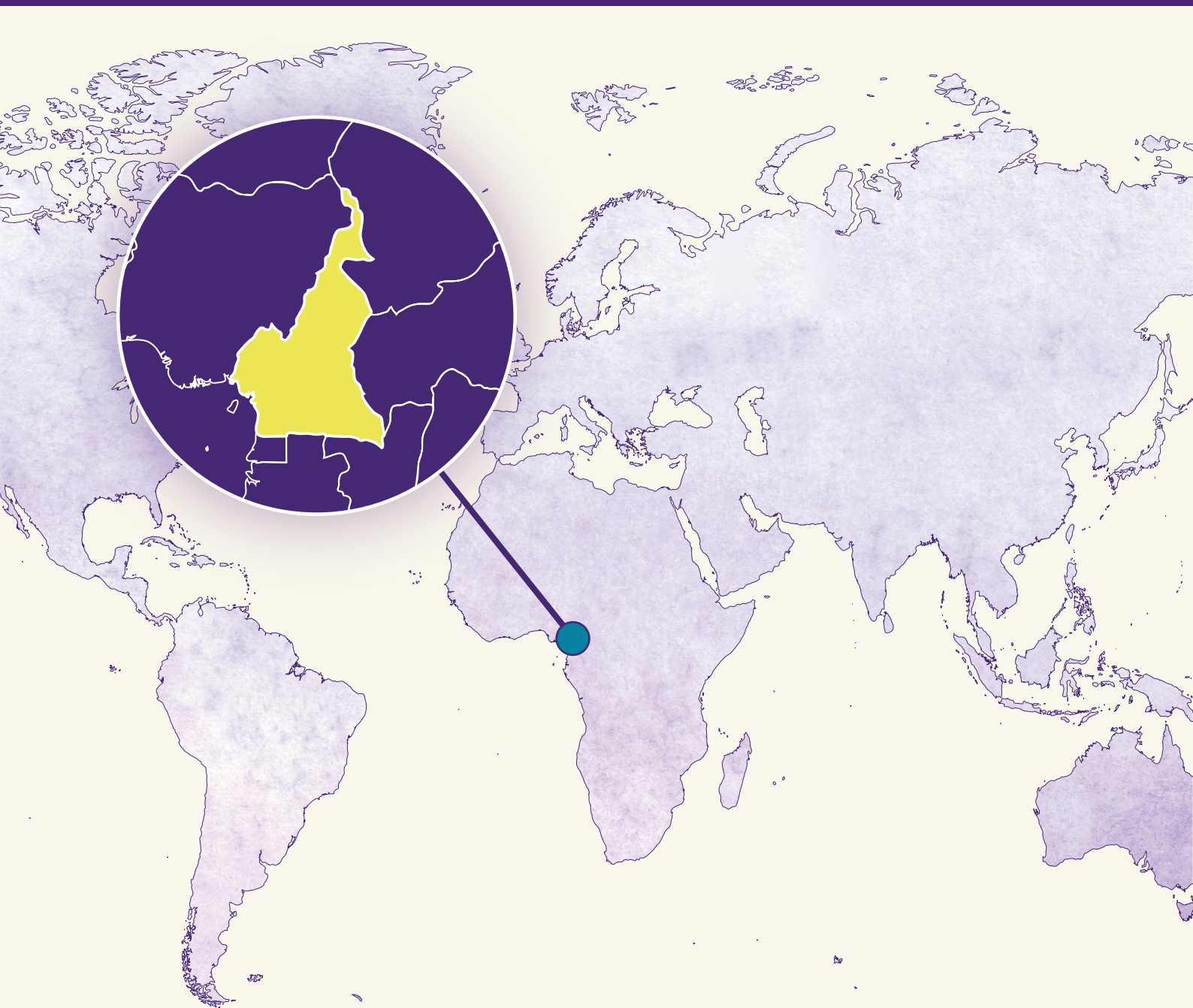


CAMEROON

Violence Against Health Care in Conflict 2025



**SAFEGUARDING
HEALTH
IN CONFLICT**



**Insecurity
Insight**
Data on People in Danger



REPORTED INCIDENTS AND MOST COMMONLY REPORTED CONCERNS

 REPORTED INCIDENTS	 HEALTH WORKERS KIDNAPPED	 HEALTH WORKERS ARRESTED
2025		
30	18	9
2024		
48	19	21
2023		
34	6	7

Source: Insecurity Insight for the SHCC.

OVERVIEW

Using data collected by Insecurity Insight, the Safeguarding Health in Conflict Coalition (SHCC) identified 30 incidents of violence against or obstruction of health care in Cameroon in 2025, compared to 48 in 2024 and 34 in 2023. Among incidents reported in 2025, 18 health workers were kidnapped and nine arrested.

The actual number of incidents and the severity of the problem are likely greater, because attacks on health care may not have been reported in some locations and conflict-related service closures may not have been reflected in the data, while cuts to United States Agency for International Development (USAID) funding further reduced the presence of health care providers and the operation of health facilities.

Methodology and sources

Information on incidents of violence against health care in Cameroon is compiled from open sources, aid agency data-sharing mechanisms and information projects. Data is continuously updated and numbers may change if new information is made publicly available. For the latest available figures, see Insecurity Insight's [global map](#) on attacks on health care. See [Methodology](#) for further information.



THE CONTEXT

Protracted non-international armed conflicts continued in Cameroon in 2025 between the Cameroon Armed Forces (CAF) and non-state armed groups. These included Ambazonian separatists in the anglophone Northwest and Southwest regions, and Boko Haram and Islamic State West Africa Province (ISWAP) in Far North region, with hostilities reported across these areas. Members of the anglophone separatist group Bui Unity Warriors were involved in fighting with state forces in Northwest's Bui department during 2025.

Following October's presidential election, in which official results declared incumbent President Paul Biya the winner despite opposition claims of his defeat, security forces responded to opposition-led protests with lethal force and mass arrests, resulting in civilian deaths and detentions being reported in several parts of the country.

Cameroon continued to face three overlapping humanitarian crises in 2025: the Lake Chad Basin conflict, the anglophone crisis in Northwest and Southwest regions, and the presence of refugees from the Central African Republic and Nigeria. Despite some reductions, millions of people remained in urgent need of assistance: at the start of 2025, 3.3 million people required life-saving assistance, including nearly 2.5 million facing acute food insecurity. By December, 2.9 million still needed support, with 1.8 million targeted for aid. Displacement increased in parts of Northwest and Southwest regions during periods of intensified clashes and lockdowns, when thousands reportedly fled due to insecurity.

USAID funding freeze disrupted humanitarian and health programmes in Cameroon. Cuts to the US-funded President's Malaria Initiative reduced supplies of essential antimalarial drugs in Far North region, where malaria mortality had previously declined by nearly 60% between 2017 and 2024. The suspension of USAID support also led to the termination or scaling down of programs in education, livelihoods, peacebuilding and humanitarian relief in the Lake Chad Basin. Funding cuts also disrupted HIV services, including testing and the provision of paediatric antiretrovirals, raising concerns about higher HIV transmission rates and reduced access to care.

Humanitarian access in Northwest and Southwest regions remained restricted. Non-state armed groups continued to enforce "ghost town" days on Mondays and imposed repeated lockdowns during which humanitarian activities were suspended. Weak health care systems, limited access to quality care, and recurring outbreaks of diseases such as cholera and measles continued to strain health responses, particularly in remote and underserved communities affected by displacement and climate-related shocks. Cuts in international aid in 2025 strained the humanitarian response capacity, placing additional pressure on health and protection services.

VIOLENCE AGAINST OR OBSTRUCTION OF HEALTH CARE IN 2025

In 2025, incidents of violence against or obstruction of health care in Cameroon were reported in three regions, down from four in 2024. As in previous years, most occurred in the anglophone Northwest and Southwest regions, where armed clashes continued, including between Ambazonian separatists and the CAF, although the overall number of cases in these regions declined compared to 2024. Incidents of reported violence against health care providers in Far North region more than halved between 2024 and 2025, possibly reflecting the reduced presence of health providers due in part to USAID funding cuts.



Most incidents involved the use of rifles. In Far North region, ISWAP deliberately set a health center on fire during an attack on three surrounding villages in Mayo-Tsanaga department, while Boko Haram set fire to another health center in Logone-et-Chari department.¹

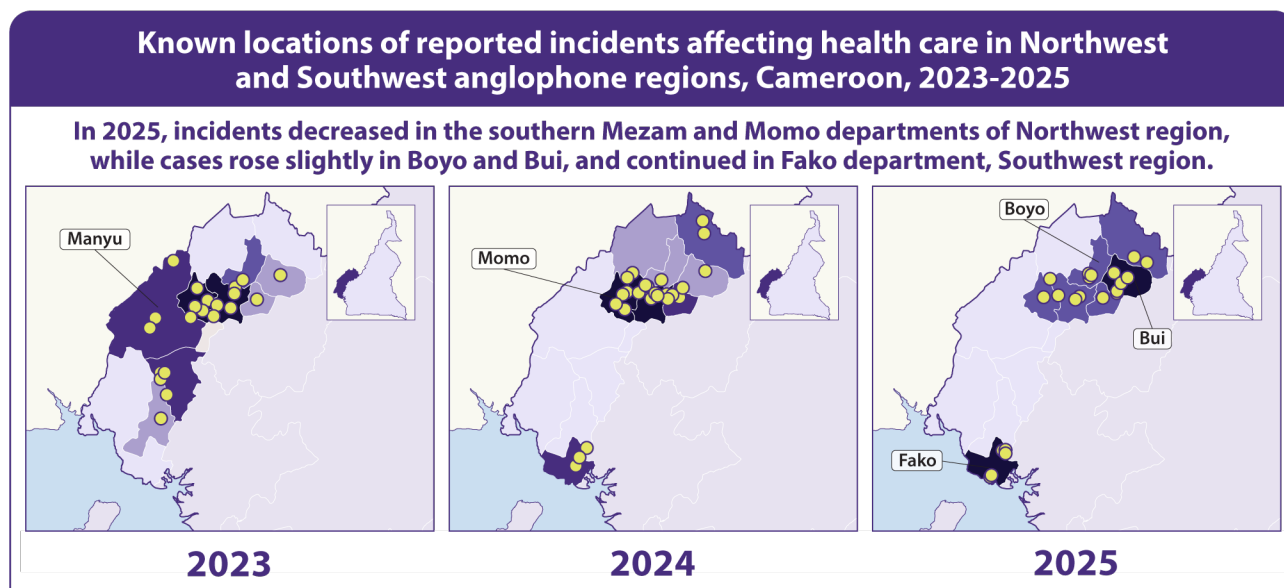
For the second consecutive year, reported health worker kidnappings in Northwest region increased, with Ambazonian separatists and Bui Unity Warriors often named as perpetrators. Gendarmerie forces in Northwest and Southwest regions continued to arrest health workers, accusing them of links to opposition groups.

The looting of medical supplies by Boko Haram, ISWAP and other groups persisted in Far North region, although at lower levels than in previous years. In some incidents, the responsible conflict party remained unidentified.

Over half of incidents occurred at health facilities, a trend consistent with previous years. Health workers were mostly harmed while on their way to or from work, at home, or pursuing a private activity, highlighting how their vulnerability is heightened in their non-working lives within the community. The majority of incidents affected health care providers operating in national health structures, with one incident affecting a private health organization.²

Northwest and Southwest anglophone regions

In total, 22 incidents of violence against or obstruction of health care were reported in the anglophone regions in 2025, compared to 30 in 2024 and 25 in 2023. In 2025, incidents decreased in the southern Mezam and Momo departments of Northwest region, while cases rose slightly in Boyo and Bui, and continued in Fako department, Southwest region.



Source: Insecurity Insight for the SHCC.

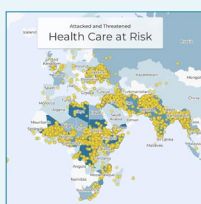
At least 16 health workers were reported kidnapped in nine incidents in Northwest region in 2025, compared to 16 in five incidents in 2024 and five in six incidents in 2023. Health worker kidnappings were mostly reported in Bui and Ngoketunjia departments, marking a change from Mezam department in 2024. Separatists and unidentified attackers armed with rifles kidnapped health workers at illegal checkpoints while they were traveling to remote areas to provide care, and from health centers and their homes.



Some kidnapped health workers were released after paying their captors ransoms ranging from XAF 5,000 to XAF 10 million (EUR 10-15,000). Other health workers were kidnapped and accused by their kidnappers of collaborating with opposing forces.³ In a separate case in January, a health worker was kidnapped at a checkpoint by the Bui Unity Warriors faction on accusations of being a teacher. He was released hours later after negotiations.⁴ The fates of the remaining kidnapped staff were not recorded.

Between May and June, nine health workers were reported arrested by Cameroonian police in six incidents in 2025, compared to 19 in four incidents in 2024 and seven in four incidents in 2023. Health workers were arrested in Southwest's Fako department and Northwest's Boyo, Momo, and Donga-Mantung departments on accusations of collaborating with or providing medical assistance to opposition forces. Cameroonian police also entered health centers in Fako department in search of members of opposition forces, disrupting care.⁵

In other cases, health workers were reportedly assaulted at checkpoints or in their homes, often over allegations of refusing to treat armed group members or as part of armed-group-imposed taxes. Ambulances were extorted during the enforcement of separatist-imposed lockdowns.⁶



This chapter is based on 2023-2025 CMR SHCC Health Care Data. Download this data on the [Humanitarian Data Exchange \(HDX\)](#).

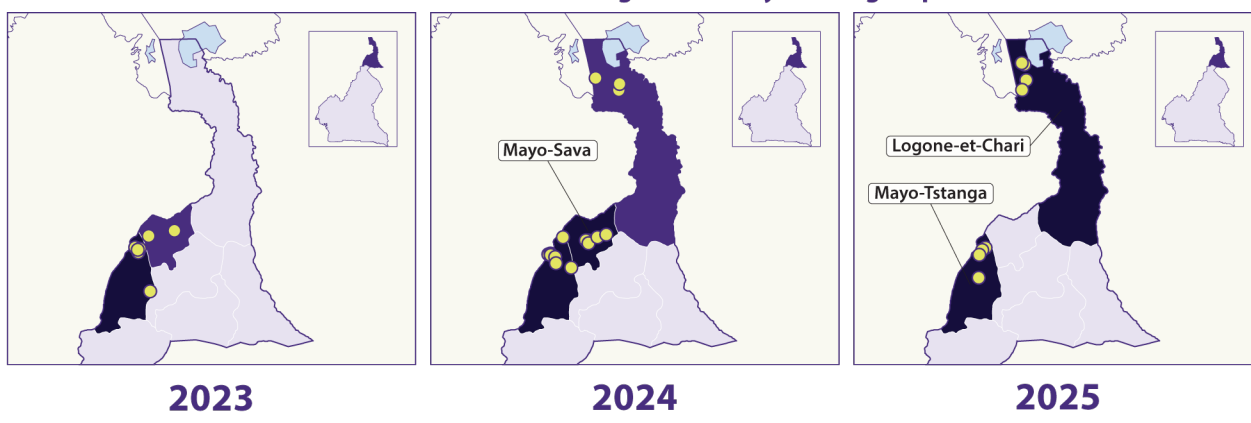
Insecurity Insight updates data continuously and numbers may change if new information is made publicly available. For the latest available figures, see Insecurity Insight's [global map](#) on attacks on health care.

Far North region

Eight incidents of violence against or obstruction of health care were recorded in Far North region, compared to 18 in 2024 and nine in 2023. Overall, incidents in Mayo-Tsanaga department decreased, although they still accounted for 50% of all incidents reported in the region. Cases also decreased in Mayo-Sava department from five in 2024 to zero in 2025.

Known locations of reported incidents affecting health care in Far North region, Cameroon, 2023-2025

Most incidents involved health centers being attacked by armed groups armed with rifles.



Source: Insecurity Insight for the SHCC.



The majority of incidents involved health centers being attacked by armed groups armed with rifles, primarily Boko Haram and ISWAP. Vital medical supplies, including vaccines, medicines, and equipment, as well as solar panels and roofing materials were some of the supplies looted. Health centers were also vandalized or destroyed, with fires being deliberately set during assaults on villages. Attacks on health facilities often occurred alongside wider acts of violence, including civilian abductions and killings, livestock raids, and at least one civilian killing.

ISWAP fighters armed with rifles reportedly abducted two health workers and two children from their homes after the group withdrew from the area following armed clashes with Cameroonian forces at a security post in Logone-et-Chari department.⁷

THE IMPACT OF ATTACKS ON HEALTH CARE

Repeated violence and insecurity in Northwest, Southwest and Far North regions continued to affect the functioning of health services in 2025. Lockdowns enforced by non-state armed groups and movement restrictions linked to armed clashes limited health workers' and patients' ability to travel to obtain health care and disrupted routine preventive health care activities. Insecurity, movement restrictions, and access constraints during lockdown periods forced humanitarian partners to suspend or adapt field operations, resulting in operational delays and reduced capacity to implement activities in certain areas and constraining the delivery of medical assistance.

Clashes between non-state armed groups and state security forces, including ambushes and improvised explosive device (IED) attacks, resulted in civilian casualties, mass arrests, and new displacement in several localities, leaving displaced populations with limited shelter and limited availability of essential services, including health care. Communities in remote and underserved areas continued to face challenges when attempting to access essential health services, including fragile infrastructure, recurrent disease outbreaks such as malaria and yellow fever, and displacement linked to conflict- and climate-related shocks.

Amid widespread humanitarian needs and acute food insecurity, gaps in the health care system, staff shortages, and uneven services caused high levels of preventable illnesses and mortality, particularly among children and pregnant women. Humanitarian access constraints and insecurity affecting both civilians and aid providers shaped the health care operational environment, limiting where and how services could be delivered.

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- 1 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident numbers 118781; 110133.
 - 2 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident number 122257.
 - 3 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident numbers 122271; 122273.
 - 4 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident number 122259.
 - 5 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident numbers 122262; 122268.
 - 6 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident number 118794.
 - 7 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 CMR SHCC Health Care Data. Incident number 123606.

SAFEGUARDING HEALTH IN CONFLICT

The Safeguarding Health in Conflict Coalition is a group of more than 40 organizations working to protect health workers and services threatened by war or civil unrest. We have raised awareness of global attacks on health and pressed United Nations agencies for greater global action to protect the security of health care. We monitor attacks, strengthen universal norms of respect for the right to health, and demand accountability for perpetrators. Suggested citation: Safeguarding Health in Conflict Coalition (SHCC) 2026. Care in the Crosshairs: Violence Against Health Care in Conflict, May 2026. <https://shcc.pub/SHCCAnnualReport2025>
<https://safeguarding-health.com>

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